Alectinib monotherapy

Confirm your alectinib treatment setting This guide covers alectinib alone for ALK-positive lung cancer. Confirm whether it is adjuvant treatment after complete surgery, first treatment for advanced cancer, or treatment after crizotinib. These settings have different evidence and treatment lengths.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Alectinib daily oral treatment

Active medicines: alectinib

Emetogenic risk: Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Alectinib is minimal-to-low oral emetic risk. Treat nausea with prescribed as-required medicine if needed. Routine preventive anti-sickness treatment and infusion premedication are not required; your team selects a suitable rescue medicine.
- The usual starting dose is 600 mg twice daily: four 150 mg capsules each time, taken with food. Use your own prescription, which may specify a lower dose. Swallow capsules whole; do not open or dissolve them. If a dose is missed, take it only if the next dose is more than 6 hours away. If the next dose is due within 6 hours, skip the missed dose. Do not take two doses together. If you vomit after a dose, take the next dose at its usual time; do not take an extra dose.
- Keep liver, creatine phosphokinase (CPK) and blood-count appointments. Muscle symptoms, jaundice, dark urine, unusual fatigue, cough or breathlessness need prompt review even between appointments.
- Tell the team about dizziness, fainting and medicines that slow the pulse. Check new prescriptions, supplements, grapefruit and Seville oranges with pharmacy; interactions may require closer monitoring rather than an automatic dose change.
- Avoid prolonged sunlight during treatment and for at least 7 days after stopping. Use broad-spectrum UVA/UVB sunscreen and lip balm sun protection factor (SPF) 50 or higher.
- Women who could become pregnant need highly effective contraception during treatment and for at least 5 weeks afterwards. Men with female partners who could become pregnant need highly effective contraception during treatment and for at least 3 months afterwards. Oral contraceptive effectiveness may be reduced: ask for a suitable method. Do not breastfeed during treatment. Discuss fertility before treatment and contact the team promptly if pregnancy occurs.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00401 v6 · p4; oral classification Tables 5 and 8 · NCCP oral minimal–low risk: as-required antiemetics only; NCIS V8 checked separately

Sources checked 2026-10-29 · SACTCheck v0.84.24 · Regimen-level drafts; prescriber and oncology-pharmacy verification required